

Prior Authorization Request

Health Plan

Humana Medicare Advantage -- Gold Choice PPO.

Member Information

Member: Benedict A. Thorvaldsen DOB: 1961-12-30 (age 64) Member ID: MBR-71206

Sex: M

Requesting Provider

Practice: Northgate Heart Associates

Requested Service

Requested service: Elective inpatient admission, implantation of a transvenous implantable cardioverter defibrillator (ICD)

CPT 33249

Planned admit date: 2026-10-21

Level of care requested: INPATIENT

Made-up data for a software prototype. Not a real patient.

Cardiac catheterization report

NORTHGATE MEDICAL CENTER - CARDIAC CATHETERIZATION

Date: 08/06/2026 Pt: Thorvaldsen, Benedict A. 64 M

Indication: new cardiomyopathy, LVEF ~20% on TTE, r/o ischemic etiology

Coronary angiography:

LM: normal

LAD: 20% proximal plaque, otherwise smooth

LCx: normal

RCA: dominant, luminal irregularities <20%

LV gram not performed (Cr).

Conclusion: No angiographically significant CAD. No culprit lesion. Findings do not support an ischemic etiology.

No intervention.

Operator: F. Okonjo, MD

Made-up data for a software prototype. Not a real patient.

Cardiac MRI report

Northgate Imaging - CARDIAC MRI WITH/WITHOUT GADOLINIUM

Exam: 08/07/2026 Patient: THORVALDSEN, BENEDICT

LVEDVi 142 mL/m². LVEF 22%. RVEF 41%.

Late gadolinium enhancement: linear mid-wall LGE in basal-mid septum. No subendocardial or transmural LGE; negative for infarct pattern in any coronary territory.

T2: no edema. No findings of myocarditis, sarcoid, or amyloid.

Impression: Dilated non-ischemic cardiomyopathy with septal mid-wall fibrosis. No evidence of prior MI.

Made-up data for a software prototype. Not a real patient.

Hospital discharge summary

Northgate Medical Center - DISCHARGE SUMMARY

Admission: 08/05/2026 Discharge: 08/09/2026

Thorvaldsen, Benedict A.

Dx: new HFrEF, NICM. Type 2 demand-related troponin elevation (hs-TnT 38->41->39, flat) - not MI, no ACS.

Pt denies any prior MI or chest pain. No Q waves on ECG.

Diuresed 6 L. Cath and CMR as reported.

GDMT started on day of discharge (08/09/2026): sacubitril/valsartan 24/26 BID, carvedilol 3.125 BID, spironolactone 12.5 daily, empagliflozin 10 daily. Lisinopril for HTN was stopped on admission and not resumed.

Follow-up: HF clinic 1 wk, EP referral.

Made-up data for a software prototype. Not a real patient.

Echocardiogram report

NORTHGATE HEART ASSOCIATES - TTE

Study date 09/30/2026

Thorvaldsen, Benedict DOB 12/30/1961

LVEF 25% (biplane). Global HK. LVIDd 6.8.

RV normal. Mild MR. No thrombus.

Impression: Severe LV dysfunction, minimal change from 08/2026 CMR (22%).

Made-up data for a software prototype. Not a real patient.

EP consultation

Northgate Heart Associates - EP CONSULT

09/30/2026 Ref: HF clinic

Benedict Thorvaldsen, 64M

HPI: NICM dx'd 08/2026. On GDMT since that admission, titrating. Short of breath doing yard work and walking a few blocks. Denies syncope. Denies chest pain. No family hx of SCD. Daughter is moving him to her home out of state in November and he wants the device placed before the move.

Assessment: NICM, LVEF 25%, NYHA III.

Comment: guideline preference is a longer GDMT period before ICD for NICM; pt informed. Pt and family prefer to proceed; will request auth.

Plan: SDM completed today with HF NP (separate note). Book ICD 10/21.

R. Lindahl-Moss, MD

Made-up data for a software prototype. Not a real patient.

Shared decision making note

SDM - ICD 09/30/2026 Northgate HF clinic

Pt: Benedict Thorvaldsen Daughter present by video

NP: Yvette Marchand, AGACNP

Used evidence-based ICD decision aid (printed version), walked through benefit/harm pictograms, shock experience, infection, battery changes, deactivation. Discussed waiting for EF recovery on GDMT as an option and wearable defibrillator as a bridge.

Pt chooses ICD now. Understanding confirmed with teach-back.

Y. Marchand NP

Made-up data for a software prototype. Not a real patient.

Admission plan

PRE-ADMIT FORM - EP LAB

Pt: Thorvaldsen, Benedict A. Admit 10/21/2026 Inpatient CPT 33249

Expected LOS: 2 midnights

Comorbid factors: DM2 on insulin (A1c 9.0), BMI 41, HTN, CKD 3a w/ contrast for venogram.

Contraindication screen: cognitively intact, no terminal dx, sinus rhythm (no AF), stable as outpatient - none.

No history of MI, PCI or CABG.

Signed R. Lindahl-Moss MD 10/01/2026

Made-up data for a software prototype. Not a real patient.